

Addendum B: PDF and Provider Page Revision Spec

****August 3, 2026 | Internal Document | Not for Distribution****

Exact replacement copy for the two downloadable PDFs and the affected provider component. Decisions settled in discussion; this is the build sheet.

SETTLED ECONOMICS

The chain, reconciled:

Value
Center per-scan reimbursement, plain MRI **\$225-\$400** , varies by market
USRad fee, flat per modality **\$75** (plain MRI)
Published global price to patient or employer **\$300-\$475**
Volume basis **15-25 additional scans/month**
Realistic monthly add to center **\$3,400-\$10,000**

Every figure in this spec derives from these five lines. Nothing else in either document may state per-scan or monthly economics independently.

****Rules now in force:****

1. ****"Per-scan reimbursement," never "net margin."**** USRad cannot know a center's net margin. It knows what it pays.
2. ****Plain MRI only.**** No "avg across modality mix" language until the full flat-fee table is finalized.
3. ****15-25 scans/month is the single volume basis.**** No component may imply a different one.
4. ****The incremental argument goes in prose, not in a number.****

1. PROVIDER OPPORTUNITY BRIEF

1.1 Page 1 – revenue tiles

****Replace all three tiles and the footnote.****

Tile	Current	Replace with
Left	**\$375** / NET MARGIN PER SCAN / avg across modality mix	**\$225-\$400** / PER-SCAN REIMBURSEMENT / plain MRI, varies by market
Center	**+15-25** / ADDITIONAL SCANS/MO / from existing capacity	<i>*unchanged*</i>
Right	**\$5K-\$10K** / REALISTIC MONTHLY ADD / at 15-25 scans/month	**\$3,400-\$10,000** / REALISTIC MONTHLY ADD / at 15-25 scans/month

****Footnote – replace in full:****

> * Per-scan reimbursement reflects contracted rates for plain MRI and varies by market. Each center sets its own rate, subject to USRad approval, and receives that rate on every assignment. Monthly figures assume 15-25 additional scans per month. Results vary.

Removes the modality-mix claim, states that the center sets the rate, and gives the range a stated basis.

1.2 Page 1 – the incremental margin argument

****Add below the tile row****, as prose rather than a figure:

> ****Why incremental volume matters.**** When a scanner has open capacity, an additional completed scan adds revenue without a matching increase in equipment, occupancy, or administrative overhead. Each center can evaluate the contribution against its own read fees, staffing, and supply costs.

This is the argument the \$375 figure was trying to make. In prose it is verifiable by the center against its own numbers and cannot be arithmetically falsified.

****Revised from an earlier draft**** that read "your fixed costs are already covered... the tech time, equipment, and overhead are already paid for whether the slot runs or sits empty." That version asserted a cost structure USRad cannot see. Tech time is not reliably fixed where overtime, per diem, or flexed staffing is involved, and not every center's fixed costs are covered in every operating period. The revised language makes the same argument without stating the center's economics for them – and inviting the center to run its own numbers is more persuasive than telling them the answer.

1.3 Page 1 – headline

Current	Issue	Replace with
Turn Empty Scanner Slots Into **Guaranteed Monthly Revenue.**	"Guaranteed" attaches to <i>*monthly revenue*</i> , implying a volume commitment. Contradicts <code>`FoundingPartners.astro:327`</code> – "No volume commitments"	Turn Empty Scanner Slots Into **Predictable Revenue.**

The 10-day payment guarantee is defensible and stays. A revenue guarantee is not.

1.4 Page 1 – subhead

Current	Replace with
USRad delivers **pre-paid, price-confirmed patients** directly to your imaging center. No insurance. No billing complexity. Payment issued within 10 days of fulfillment.	USRad delivers **pre-funded, price-confirmed assignments** directly to your imaging center. No insurance. No billing complexity. Payment issued within 10 business days of fulfillment.

Also corrects "10 days" to "10 business days," matching the guarantee box below it and PSA § 4.3.

1.5 Page 1 – guarantee box

Line	Current	Action
1	Payment issued within 10 business days of scan fulfillment.	Keep
2	**Every assignment represents a patient who has already agreed to the price.**	Replace: **Every assignment is funded before it is routed to your center.**
3	No denials, no chargebacks, no collections. Revenue is confirmed before the patient arrives.	**Revise:** No insurance denials, no patient collections, no claims follow-up. Funding is confirmed before the patient arrives.

> ****Why "chargebacks" comes out.**** It is a card-industry term covering a broader set of events than insurance adjudication. Patient card disputes, fraud reversals, duplicate payments, and refunds remain possible under any model, and USRad cannot promise otherwise. "No insurance denials, no patient collections, no claims follow-up" states the actual benefit precisely and does not make an absolute treasury claim. "Revenue" also becomes "funding," since the sentence is about the assignment being funded rather than about the center's revenue recognition.

1.6 Page 1 – section header and bullets

Current	Replace with
WHY **CASH-PAY** IMAGING WORKS FOR YOUR CENTER	WHY **PRE-FUNDED** IMAGING WORKS FOR YOUR CENTER

Bullets: all eight are funding-neutral and stay as written.

1.7 Footer – both pages

Current	Replace with
usrad.com \ U.S. Radiology of Florida LLC \ National **Cash-Pay** Imaging Infrastructure usrad.com \ U.S. Radiology of Florida LLC \ National **Pre-Funded** Imaging Infrastructure	

Appears on every page directly under the legal entity name. Strongest instance of cash-pay-as-identity in any artifact.

1.8 Page 2 – portal flow, Step 1

Six of seven steps are funding-neutral and stay as written. Step 1 only:

Current	Replace with
Patient Books & Pays – Patient selects your center, confirms price, and completes payment through the USRad platform. **Assignment Funded** – The patient or their employer completes funding through the USRad platform. Funding is confirmed before the assignment is routed.	

1.9 Page 2 – credibility figures

Current	Replace with	Reason
30 Years / Imaging industry experience – AnciCare founded 1994 **1994-2002** / AnciCare – founded, operated, and sold to CorVel	Conflates career span with company operating life. The dates are the fact	
1,200+ / Imaging centers successfully recruited – \$180M+ in volume **1,200+** / Imaging centers recruited – **over \$150 million** in provider volume	Center count is documented and stays precise.	

Volume is extrapolated; round down. Also resolves the \$180M vs `TrustBar`'s \$160M conflict |
 | ****168,000**** / Patients served across our prior imaging network | ****150,000**** / Patients served across
 our prior imaging network | Five significant figures on an extrapolation. Rounded down, no less
 impressive, defensible under questioning |

****Site conforming changes:**** `TrustBar.astro:77-80` (\$160M+), `ProvenSuccess.astro:17-29` (\$180M),
 `AnciCareLegacy.astro:96`, `CredibilityBar.astro:26`, `FinalCTA.astro:103` – all must state the same
 rounded figures.

1.10 Page 1 – layout

Trailing whitespace measures approximately 21%, not the 40% previously stated. Correction noted.

At 21% this is noticeable but not broken. If tightened, the fix is distributing space through the four
 sections above and moving the CTA block toward the bottom margin – not adding content. ****Lowest priority
 item in this spec.****

2. EMPLOYER ROI REPORT

Generated template via `/api/employer-roi-report`. All changes are code. ****Execute as a single pass**** –
 Release 1 and Release 2 items together, since the compliance fixes and the rate language describe the
 same commercial terms and splitting them ships an incoherent intermediate.

2.1 Rate language – pages 1, 2, 3

Current	Replace with
USRad **flat rates** (\$350) USRad network rates	at **\$350 flat rate** /scan at network rates, varies by market

The document already contradicts itself: page 3's table shows MRI at \$260-350 and CT at \$250-350. ****The
 table is correct.**** Make the headline consistent with the table already present in the same document.

2.2 Compliance block – page 5, replace in full

Current	Replace with
ERISA Compliant – Structured as a permissible supplemental benefit – no plan amendments required **Plan Structure** – Designed as a supplemental benefit that operates alongside your existing plan. Your benefits counsel should confirm fit with your plan documents **HIPAA / SOC 2 Type II** – Full data security certification maintained **Data Security** – HIPAA- compliant PHI handling protocols and Business Associate Agreements with all network participants **Licensed All 50 States** – Workers' comp imaging approved in every jurisdiction **Network Coverage** – Contracted imaging centers across [N] states, all facility-accredited and independently licensed **TPA Integration** – Works with Sedgwick, Gallagher Bassett, ESIS, Broadspire + all major TPAs **Direct Contracting** – USRad contracts directly with your organization. No carrier or TPA intermediary required **Zero Network Conflicts** – Supplemental overlay Keep	

Four changes: removes a legal conclusion, removes a certification that does not exist, removes a
jurisdictional approval claim for a deferred product, and replaces carrier positioning with direct-to-
employer.

****If a SOC 2 Type II report actually exists,**** state it separately and precisely – "SOC 2 Type II report
available on request." Do not pair it with HIPAA.

2.3 Implementation roadmap – page 5

Week	Current	Action
1	· Execute master services agreement Keep, and **add funding bullet** – see 2.4	
1	· Review historical imaging spend data Keep	
1	· Map existing WC & health plan workflows Replace: · Map covered population and eligibility data	
1	· Identify implementation stakeholders Keep	
1	· **Finalize TPA integration requirements** **Delete**	
2-3	· Configure employer portal & booking Keep	
2-3	· **Coordinate with benefits platform / TPA** **Delete**	
2-3	· **Establish claims billing & reporting feeds** Replace: · Establish utilization reporting	
2-3	· Complete integration testing & validation Keep	
2-3	· Activate network in employee geography Keep	
4	· Deploy employee communications Keep	

4	• **Train HR team & WC adjusters**	Replace: • Train HR and benefits team
4	• Activate employer dashboard	Keep
4	• Go-live with dedicated support	Keep
4	• Review early utilization baselines	Keep

2.4 Funding language – page 5, Week 1

****Add as a new bullet under "Execute master services agreement":****

> • Establish program funding arrangement

This is the employer-side mirror of ``Implementation.astro:50`` and it is the change that makes the rest of the document coherent. Same language should be written into the page component in the same pass.

2.5 Savings figures – internal consistency

The document states four different savings rates about the same program.

Location	Figure
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p.1, p.2, p.3 headline	85%
p.4, Day 1	50-70%
p.4, Month 3	40-60%
p.5, historical	50-70%

****Resolution:**** the headline is the computed figure and stays, ****but must be qualified to the modeled comparison**** – "up to 85% below the illustrated hospital outpatient baseline," not a bare 85%. Unqualified, a reader takes it as expected average savings across all imaging, which it is not.

The Day 1 and Month 3 lines must be conformed to the headline or restated as ranges that contain it. The p.5 figure is AnciCare historical and should be labeled as such.

****Baseline inconsistency:**** the headline uses \$2,400 average hospital rate; page 3's table shows \$2,400-3,200 and the bar chart anchors at \$3,200. Pick one baseline and use it in all three places.

2.6 Unsupported claims

Location	Current	Action
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p.3	"USRad is the only solution that addresses both populations"	Delete "only." Superiority claim, unsubstantiated
p.4	"Average 73% increase in necessary imaging completed"	Source or delete
p.4	"18+ day reduction in average disability duration"	Source or attribute to AnciCare experience
p.1, p.5	"Founded the managed imaging category"	Soften: "an early operator in managed imaging"

2.7 The upward-only hedge – keep, but ground it

"Actual total savings typically run 20-40% higher" appears twice with nothing pointing the other direction.

The underlying claim is sound and comes from operating experience: faster imaging means faster return to the referring physician, which means fewer indemnity days, and indemnity savings frequently exceed the imaging cost itself.

****Attribute it rather than asserting it as a general rule:****

> Based on AnciCare client experience, indemnity and productivity savings frequently exceeded the imaging cost reduction itself. Those figures are excluded here.

Also add one downward acknowledgment so the hedges are not unidirectional – actual results depend on utilization and redirection rates, which the footer disclaimer already says and which should appear near the projection rather than only at the end.

2.8 Contact

``(888) USRad24`` → ****` (866) USRad24`****. Template fix.

``app.usrad.com/signup`` confirmed correct.

2.9 Workers' comp

Per D4, de-emphasize without dismantling. The WC math, the 120-scan input, and the savings column stay. Remove availability and approval claims only – the compliance block item in 2.2 and the adjuster training item in 2.3 cover this.

****Add a label to the WC savings column (p.3) and the WC output tile (p.1):****

> Illustrative future expansion scenario – not included in the current USRad employer offering.

Live WC math sitting next to copy describing WC as future expansion is confusing to a prospect. The label resolves it at no structural cost, which is the cheapest available fix and avoids the calculator rebuild the plan has parked.

3. PROVIDER PAGE – ScannerUtilization

3.1 The revenue figure

`provider/ScannerUtilization.astro:145-150`

Current	Replace with
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\$37,500/month	* Based on average cash-pay MRI reimbursement across modality mix and U.S. markets
\$3,400-\$10,000/month	* Based on 15-25 additional plain MRI scans per month at contracted per-scan reimbursement of \$225-\$400. Rates vary by market. Results vary

\$37,500 implied 100 scans/month at \$375. Under the settled economics and the 15-25 scan basis, the same tile becomes \$3,400-\$10,000.

****This is a 75-90% reduction in the provider page's headline revenue claim.**** It follows directly from standardizing on 15-25 scans/month and from correcting the per-scan figure. Two things make it the right call: it now matches the brief, so a center reading both sees one number; and it is a floor USRad can beat, which is the underpromise posture.

3.2 Other ScannerUtilization items

Line	Item	Action
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:418	"Pre-screened. Pre-paid."	→ "Pre-screened. Pre-funded."
:320-347	Day 0 → Day 10 two-lane timeline	Keep. True under prefunding
:346	"zero collections risk"	Keep. True for the center under PSA § 4.3
:377-380	"10-Day Disbursement Guarantee / No claim filing"	Keep under prefunded model
:405-409	"10 / Days to disbursement / Guaranteed. Every scan."	Keep

4. WHAT THIS SPEC DOES NOT COVER

****Blocked on the flat-fee table.**** Only plain MRI is settled at \$75. Ultrasound, CT, X-ray, DEXA, mammography, and PET fees are open. No document may state modality-mix economics until that table exists.

****Blocked on the accounting decision.**** Gross vs. net revenue treatment. The AnciCare precedent (\$475 revenue, \$400 COGS) is a real fact pattern, but USRad differs on price discretion – the center sets its rate and USRad adds a fixed fee. The strongest counterweight is v6.1 Exhibit A's alternate-rate approval clause, which should be preserved and actually exercised. Accountant's read required before the Series A model.

****Out of scope, unchanged:**** all structural rebuilds, WC architecture, `AssignmentFlowDiagram`, `ROI Calculator` restructuring. Per Implementation Plan v2.

****Defect track:**** "See Full Cost Analysis" button non-functional on /employer. `ROI Calculator.astro:593` WC math defect. Both documented, neither part of this repositioning.

Addendum B to Implementation Plan v2. Supersedes Addendum A sections 2, 3E, 4B, and 4D.